

COVID PROCEDURES: AAVA MEDICAL CENTER

COVID precautions

Initiating precautions

- Patients with COVID-19 should be put on SPECIAL RESPIRATORY precautions (respirator, eye protection, gown/gloves) and appropriate signage should be displayed
- Door should remain closed
- Negative pressure isolation is not required for care of patients with COVID-19 but may be considered if frequent aerosol generating procedures are expected
 - AGP include intubation, bronchoscopy, non-invasive ventilation, tracheotomy, manual ventilation, CPR, sputum induction, dental procedures
 - Use of routine PPV e.g. CPAP is not considered aerosol generation
- Appropriate PPE is considered effective for reducing the spread of COVID-19; patients with COVID-19 can be managed on any floor and there is no need to cohort staff assignments based on isolation or disease process
- If necessary, two patients with COVID -19 on SPECIAL RESPIRATORY precautions can be cohorted together
- Patients are able to obtain routine imaging/procedures under the procedures used under AIRBORNE PRECAUTIONS (see policy 111-I); if possible, procedure should be at the end of the day and 6 air exchanges should occur before the room is used again

Discontinuing precautions

- Day of patient's onset of symptoms OR COVID-19 diagnosis (whichever comes first) should be declared DAY 0
- An inpatient can be declared COVID recovered in the following scenarios:
 - Veteran is not immunocompromised and 10 days have elapsed (day 0-day 10)
 - Veteran is immunocompromised and 20 days have elapsed
 - Veteran has documented history of COVID-19 in the 90d prior to testing and PCR remains positive
 - Veteran has high Ct value but also has either + COVID ABs or clear history of antecedent URI in the prior 90 days (see Appendix A)
- Infection control guidance is not necessary to declare patient COVID recovered if the case is uncomplicated and team or admitting ED provider is comfortable making this decision
- Primary team should change the COVID-19 banner after patient is declared recovered via "COVID STATUS ADJUSTMENTS" note in CPRS; CPRS note is not necessary if banner is updated
- Recommendations for discontinuation of isolation are different in the outpatient setting; COVID positivity should not impact discharge to the community (see Appendix B)

CLC procedures

- COVID-19 procedures in the CLC may vary from those in the acute care hospital

- Screening
 - Currently, CLC screening is done via Roche (PCR) platform weekly in all Veterans who are not COVID-recovered in the last 60d
 - All patients should have a negative COVID PCR or be declared COVID recovered prior to transfer into AAVA CLC; COVID PCR should be done in the prior 48 hours
- Surveillance
 - Patients who have symptoms consistent with COVID-19 should have a COVID PCR done as soon as possible
- Outbreak
 - If a COVID case is detected through either screening or surveillance, all other Veterans should have a COVID swab done as soon as possible
 - If this is during routine tour, recommend Roche nares swabs
 - If this is off-tour or over the weekend and Roche cannot be performed, Cepheid is an acceptable alternative
 - CLC should be closed for admissions while outbreak investigation occurs
 - Infection control will work closely with CLC leadership to coordinate repeat testing, testing for staff, visitation, and process for opening CLC
 - CLC will typically remain closed until two consecutive surveillance rounds are negative for additional cases

Cleaning

- For known COVID-19 positive inpatients or ED patients:
 - Once patient is declared COVID-recovered, the SPECIAL RESPIRATORY Precautions sign can be removed from the door and no room terminal cleaning is needed at that time
 - After a patient who was on SPECIAL RESPIRATORY Precautions is discharged, environmental services will perform a terminal clean
(2 step utilizing quarternary disinfectant, then bleach)
- For unexpectedly positive COVID-19 outpatients/ED patients:
 - If a patient is found incidentally to have COVID-19 when in ED or in outpatient setting, terminal clean will be performed only if an aerosolizing procedure took place in the area or if the patient was unmasked and had respiratory symptoms (e.g. coughing) while in the room

Employee Health

- Employees who request COVID testing either due to + symptoms or a concerning exposure should contact Employee/Occupational Health directly at 55132
- Decisions for testing and subsequent restriction from the workplace will be made by EOH staff

Food Service

- Food service will deliver patient meals to the room of patients with COVID-19 and will leave the tray outside the room; nursing care team on the floor is responsible for donning appropriate PPE and taking the tray to the patient
- Used or dirty trays should be removed from the room by nursing and should be placed in the dirty tray area for return to food service

- Disposable dining service is not recommended for patients in SPECIAL RESPIRATORY precautions

Masking

- Masking requirements for clinical staff are variable depending on the season and community respiratory virus rates; any changes to facility policy will be communicated via all-staff e-mail
- In some circumstances (outbreak), unit policy will differ from facility policy; in this case, we will ask nurse managers to communicate temporary policy to their unit
- As of this update (4/2025), masking is only required in the hemodialysis unit and in the infusion center

Notification

- Lab will notify both primary team and floor nursing supervisors of unexpected positive COVID-19 results; nursing supervisors are responsible for making sure patient is placed in an appropriate room

PPE

- Patients with COVID-19 should be managed under SPECIAL RESPIRATORY precautions
- PPE required for patients with COVID-19 includes respirator (FIT-tested N95 or PAPR), eye protection (face shield or goggles), gown, gloves
 - FIT testing is required yearly or with facial hair changes for N95 masks
- Resources for donning/doffing PPE can be accessed at [PPE-Sequence.pdf \(cdc.gov\)](#)

Sitters

- If patients with COVID-19 require a sitter, the sitter must wear appropriate PPE (including gown/gloves, respirator, eye protection) when in the patient's room

Testing

- Currently four testing modalities are available at AAVA
 - Antigen (Veritor): dry nares swab, blue top. Returns in ~1hr.
 - PCR (Biofire): NP swab, red top. Returns in 20 min-1hr.
 - PCR (Cepheid/FluVID): NP swab, red top. Returns in ~1hr.
 - PCR (Roche): nasal swab, red band tube. Batched and run 1x daily, returns 3 hours after the run is started
- Cycle threshold values are not recommended for clinical use; these are released to Infection Control for epidemiologic purposes only
- COVID-19 testing is not required for admission to the inpatient setting or to the AIMH unit

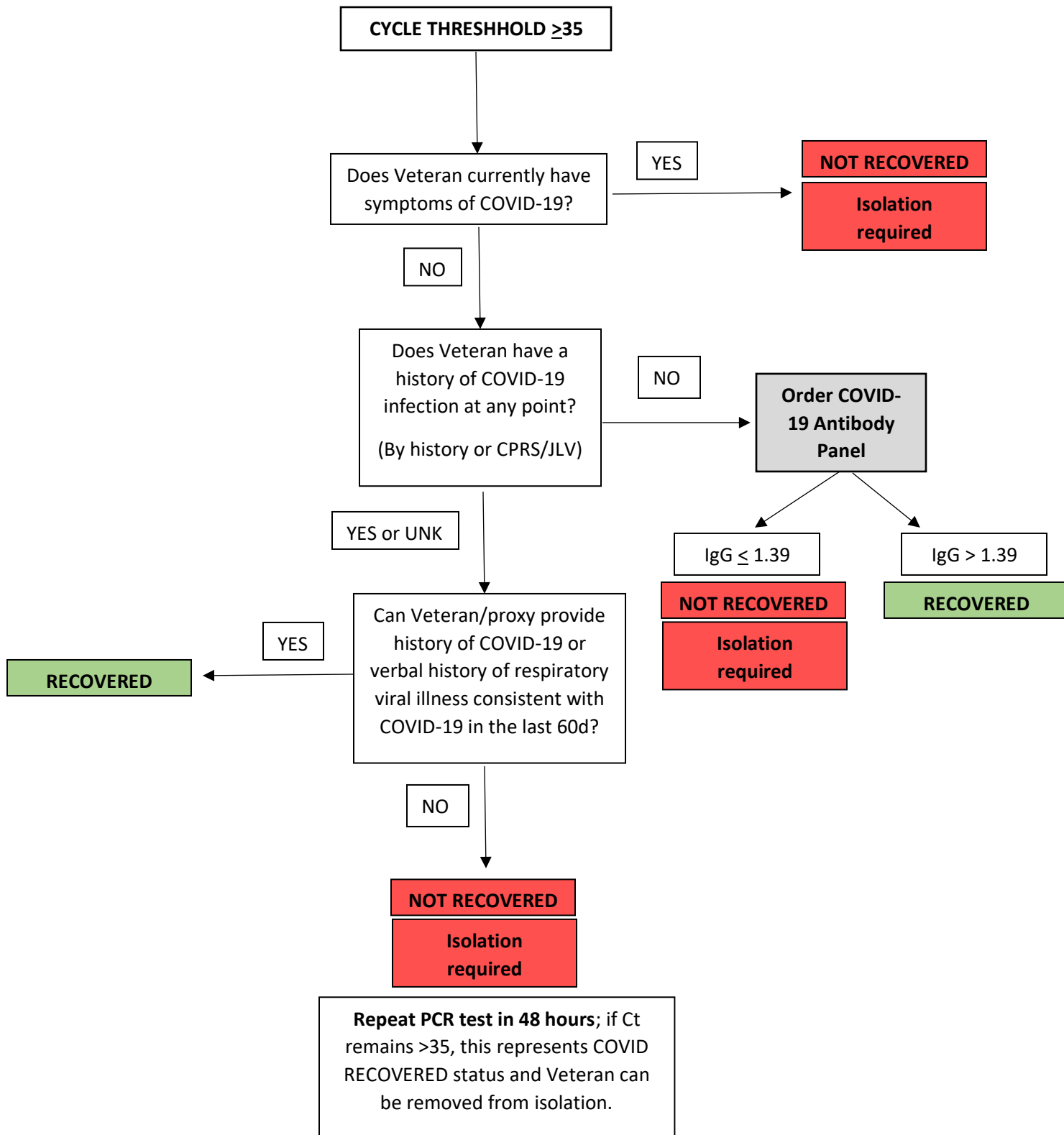
Treatment

- Recommendations for treatment of patients with COVID-19 are available in the attached Appendix C
- If advanced recommendations are needed for an individual patient, please consult Infectious Disease

Visitation

- Visitation is permitted for patients with COVID-19
- Visitors must wear an N95 (not fit-tested) or a facemask covering nose and mouth AND gown/gloves at all times when they are in the room with the patient
- Visitors must leave immediately following their visit
- If visitors are unable/unwilling to comply with these restrictions, security may be involved to escort them off the premises

APPENDIX A: DETERMINING COVID-19 RECOVERY STATUS IN VETERANS WITH HIGH Ct VALUE



APPENDIX B: Discontinuation of Isolation Precautions for COVID-19

	WHEN TO DISCONTINUE ISOLATION
Outpatients	
Asymptomatic <u>OR</u> mild illness	Isolate at home and mask around others for 5 days following initial + test. Wear a mask around others and at home and avoid people who are at risk for severe illness until day 10. <u>If you are able to test at home</u>, you may take one test after five days of isolation and another 48 hours later. If both tests are negative, you may stop isolating early. If home tests are positive, continue repeating tests every 48 hours until you have two negative tests <u>If you are not able to test at home</u>, continue to mask through day 10.
Moderate illness	Isolation and masking as above for 10 days. If 10 days have passed from symptom onset <u>AND</u> 24 hours since last fever <u>AND</u> symptoms improved isolation may be discontinued. Repeat testing is not required.
Severe illness <u>OR</u> Immunocompromise*	Isolation and masking as above for 20 days. If 20 days have passed from symptom onset <u>AND</u> 24 hours since last fever <u>AND</u> symptoms improved isolation may be discontinued. Repeat testing not routinely recommended.
Inpatients	
Asymptomatic	10 days from + test. If discharging, guidance for outpatients should be followed. If going to post-acute care facility, local infection control will determine when they allow for discontinuation of isolation precautions.
Mild/moderate illness	10 days from symptom onset <u>AND</u> 24 hours since last fever <u>AND</u> symptoms improved.
Severe illness <u>OR</u> Immunocompromise*	20 days from symptom onset <u>AND</u> 24 hours since last fever <u>AND</u> symptoms improved

** for severe immunocompromise, consider discussion with Infection Control*

Please visit CDC Website for most updated guidance: <https://www.cdc.gov/coronavirus/2019-ncov/hcp/duration-isolation.html>

APPENDIX C: Treatment of COVID-19

The type and duration of treatment is dependent on the reason for hospitalization and severity of illness. A basic way to differentiate severe from moderate infection is the new requirement of oxygen above baseline supplemental oxygen levels. The “COVID-19 Inpatient Medication Treatment” note template is a decision tool that can be used to help guide treatment. Patients in the hospital will fall into one of four categories:

1. Hospitalized WITH COVID (i.e., mild infection, withOUT increased oxygen requirements):
 - a. Nirmatrelvir/Ritonavir (Paxlovid): preferred for patients at high risk of progression to severe disease.
 - Eligible patients may include those with advanced age and high-risk comorbidities, including those who are immunosuppressed or immunocompromised.
 - FDA approved for treatment within 5 days of symptom onset.
 - Avoid with severe hepatic impairment (Child-Pugh Class C) or certain drug-interactions (see note templates for ordering or drug information resources for details).
 - To order nirmatrelvir/ritonavir: please start a new note. Note title is COVID-19 Inpatient Medication Treatment.
 - b. Remdesivir (2nd line): For patients unable to take nirmatrelvir/ritonavir, who are at high risk of progressing to severe disease.
 - Eligible patients may include those with advanced age and high-risk comorbidities, including those who are immunosuppressed or immunocompromised.
 - Treatment should be started within 7 days of symptoms onset.
 - Prior to prescribing, LFTs and PT/INR should be obtained. Remdesivir may need to be discontinued if ALT increases to >10x upper limit of normal.
 - To order remdesivir: please start a new note. Note title is Remdesivir Ordering.
 - c. Molnupiravir (Lagevrio): For patients who cannot take nirmatrelvir/ritonavir, or for whom remdesivir is not preferred.
 - Molnupiravir may result in fetal harm and is not recommended in pregnancy unless there are no other options, and therapy is clearly indicated. Sexually active males with partners who may become pregnant should also use effective contraception during therapy and for at least 3 months after the last molnupiravir dose.
2. Patients hospitalized FOR COVID withOUT increased oxygen requirements (i.e., moderate infection) one of the following could be considered in addition to supportive care:
 - a. Nirmatrelvir/ritonavir
 - b. Remdesivir
3. Patients hospitalized FOR COVID +increased oxygen requirements (i.e., severe infection):
 - a. Remdesivir 200mg on day 1, 100mg days 2-5 (See information re: Remdesivir above)

AND

- b. Dexamethasone 6mg IV or PO x10 days (or until resolution of hypoxia).

For those with rapidly increasing requirements, requiring high-flow nasal canula or non-invasive ventilation and evidence of systemic inflammation, in consultation with Infectious Diseases, one of the following could be considered in addition to the above:

- c. Baricitinib
 - d. Tocilizumab
4. Patients hospitalized FOR COVID, requiring mechanical ventilation:
- a. Treatment as above for severe infection
 - b. Consultation of Infectious Disease for consideration of baricitinib or tocilizumab